DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
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Memorandum

To: Ms. Ward Lund, Chief Executive Officer
Mr. Pope, Chief Quality Officer
From: Hon. Kevin D. Turnage, MSN Ed., RN, NHDP-BC, NPD-BC, AMB-BC, NE-BC
Subject: Alignment of Sentinel Event Policy with CMS and Joint Commission Standards
Date: Wednesday, May 21, 2025

Clarification Regarding Sentinel Events and Alignment with CMS and TJC Expectations

Thank you for continuing the discussion on how best to align our event reporting framework with federal regulatory and accreditation standards.

As the hospital's **Policy and Procedure Nurse Consultant**, I want to address an ongoing and critical concern: the absence of an explicit definition and treatment of **"sentinel events"** in the *Safety Event Reporting Policy*. This omission affects regulatory compliance, internal clarity, staff education, and ultimately patient safety outcomes.

This issue is not theoretical—it is operational and compliance-driven. More concerning, however, is the pattern of dismissing valid concerns when they do not align with prevailing internal viewpoints. The notion that if something is not of one's thought it is therefore not relevant is not only professionally inappropriate, but functionally unsustainable in a regulated healthcare environment.

1. DoD Policy as a Reference, Not a Standard

The **Chief Quality Officer has acknowledged** that the current policy was "shamelessly copied" from **Department of Defense (DoD)** policy. While I understand the appeal of using established models, it must be emphasized: **DoD policy is not a governing standard for IHS hospitals**.

DoD frameworks may offer conceptual guidance, but they do **not override the standards set by CMS, The Joint Commission (TJC), or applicable federal healthcare regulations**. As an **IHS facility**, we are held accountable to **civilian regulatory and accreditation frameworks**, including **42 CFR Part 482** and TJC standards. Our policies must reflect those expectations with specificity.

By solely modeling our reporting policy on DoD language and omitting a clearly defined sentinel event construct, we introduce inconsistency with federal expectations and increase the risk of noncompliance.

2. Why "Sentinel Events" Must Be Clearly Distinguished

This communication is intended as a protected disclosure under the Whistleblower Protection Act (5 U.S.C. § 2302(b)(8)), concerning a potential violation of law, rule, or regulation involving risks to patient safety and regulatory compliance.

The current policy refers to "reportable events" but omits the term "sentinel events," despite their well-established status in civilian healthcare:

- **Sentinel events** are defined by TJC as unexpected events resulting in **death, permanent harm, or severe temporary harm** that require immediate investigation and system-level response (TJC CAMH SE-3 to SE-6).
- Not all reportable events are sentinel. For example, a **suicide within seven days of discharge** is explicitly a sentinel event under TJC guidelines, while a **medication error without harm** may be reportable but not sentinel.

By not distinguishing sentinel events, the policy risks misclassification, under-response, and poor survey performance.

3. CMS Expectations and Regulatory Requirements

CMS, while not using the term "sentinel event" in regulation, establishes expectations that functionally align with the TJC framework:

- **42 CFR § 482.21(c)(2):** Requires hospitals to track and analyze adverse events, especially those with patient harm.
- **42 CFR § 482.21(d)(2):** Requires implementation and monitoring of performance improvement based on those events.
- **State Operations Manual (SOM), Appendix A, Tag A-0309:** Instructs surveyors to review if facilities use RCA and systemic action in response to serious harm events.
- **Appendix PP, Tag F867 (LTC):** Uses the term *sentinel event* explicitly and details expectations for systems-level corrective action.

4. IHS Status as a High-Risk Organization

We must also contextualize this discussion within our organizational accountability.

The **Indian Health Service (IHS)** has been listed on the **U.S. Government Accountability Office's (GAO) High-Risk List since 2017** due to systemic deficiencies in oversight and management of care delivery. As of February 2025, IHS has only partially met key criteria for removal, specifically lacking full demonstration of capacity, action planning, monitoring, and sustained progress.

This designation reinforces the need for immediate, standards-based policy improvements—including explicitly defining and addressing sentinel events. Failing to make these adjustments further perpetuates the risk profile that led to the GAO designation.

5. Recommendation for Policy Realignment

To resolve these issues, I recommend we pursue one of the following options:

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- **Option 1:** Amend the *Safety Event Reporting Policy* to incorporate a clear definition of **"sentinel event"** consistent with TJC and CMS frameworks, and outline required actions including RCA and corrective planning.
- **Option 2:** Create a **dedicated Sentinel Event Policy** that defines the term and formally references relevant investigative processes, corrective action mechanisms, and related policy links.

Either path would:

- Strengthen our alignment with **42 CFR § 482.21** and CMS surveyor guidance;
- Reinforce compliance with **TJC standards LD.03.09.01 and PI.01.01.01**;
- Provide clarity to staff managing high-severity events;
- Reduce the risk of improper classification and underreporting.

Conclusion

This issue is not about semantics—it is about regulatory precision and patient safety integrity. Continuing to rely on generalized or borrowed terminology when specific, nationally recognized definitions exist is neither compliant nor advisable. Given the IHS's status as a high-risk organization, we must move forward with urgency to correct this oversight and bring our policies in line with federally recognized standards.

I strongly urge leadership to recognize this distinction and act accordingly to strengthen our policy infrastructure.

what is found in generalized "reportable event" frameworks.

2. The Distinction Between Reportable and Sentinel Events

The current policy defines "reportable events" but omits any explicit reference to **"sentinel events"**, which are a defined subset of events under TJC guidance:

- **Sentinel events** involve **death, permanent harm, or severe temporary harm** requiring life-sustaining intervention. They are events that "signal the need for immediate investigation and response" (TJC CAMH, SE-3 to SE-6).
- A **reportable event** may include a range of occurrences, some of which carry no harm. But **not all reportable events are sentinel**, and **not all sentinel events are appropriately captured without a clear definition**.

Relying on broad terminology is not sufficient. Sentinel events are associated with **specific expectations** for **root cause analysis (RCA)**, **corrective action planning**, and **monitoring**, which must be clearly defined in our policy infrastructure.

3. CMS Expectations and Surveyor Guidance

CMS does not define the term "sentinel event" in the regulations, but it aligns operationally with the TJC framework:

- **42 CFR § 482.21(c)(2):** Requires hospitals to track and analyze adverse events, particularly those involving harm.
- **42 CFR § 482.21(d)(2):** Requires implementation of effective corrective actions and outcome monitoring.
- **State Operations Manual (SOM), Appendix A, Tag A-0309:** Instructs surveyors to confirm that RCAs and systemic interventions are applied to significant harm events.
- **Appendix PP, Tag F867 (LTC):** Uses the term *sentinel event* and describes expectations for systemic investigation and response.

CMS surveyors expect hospitals to demonstrate sentinel-event-level accountability for serious incidents—even when not using that specific term internally.

4. Recommendation for Policy Alignment

To correct this gap and ensure alignment, I respectfully recommend one of the following:

- **Option 1:** Revise the *Safety Event Reporting Policy* to define "sentinel event" in accordance with TJC and CMS expectations, and outline associated RCA and reporting requirements.
- **Option 2:** Develop a **dedicated Sentinel Event Policy**, formally cross-referenced with the current policy, to address all criteria, timelines, documentation, and follow-up processes.

This will:

- Support compliance with **42 CFR § 482.21**;
- Align with **TJC LD.03.09.01** and **PI.01.01.01**;
- Improve staff clarity and system responsiveness to high-severity events;
- Protect the organization from survey deficiencies or avoidable regulatory risk.

Conclusion

This issue is not about semantics—it is about regulatory precision and patient safety integrity. Continuing to rely on generalized or borrowed terminology when specific, nationally recognized definitions exist is neither compliant nor advisable. I strongly urge leadership to recognize this distinction and act accordingly to strengthen our policy infrastructure.

Respectfully,

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